

Opinion of BREYER, J.

abortions.” App. 273. And nothing in the record suggests that Christus, 10 years earlier, was aware of Doe 2’s connection with abortion. JUSTICE ALITO imagines a number of ways that Christus may have become aware of Doe 2’s or Doe 3’s abortion practice. See *post*, at 393–394, and n. 10 (dissenting opinion). The State apparently did not see fit to test these theories or probe the doctors’ accounts on cross-examination, however. And the District Court’s finding of good faith is plainly permissible on the record before us.

Finally, the Court of Appeals faulted Doe 2 for failing to apply to Minden Hospital. The record also explains that decision. Minden subjects all new appointees to “not less than” six months of “focused professional practice evaluation.” Record 9281; see also *id.*, at 9252. That evaluation requires an assessment of the provider’s in-hospital work. See *supra*, at 326–327. Doe 2 could not meet that requirement because, as we have said, Doe 2 does not do in-hospital work, and only two of his patients in the past five years have required hospitalization. App. 400. Moreover, Minden’s bylaws express a preference for applicants whom “members of the current Active Staff of the Hospital” have recommended. *Id.*, at 1211. Doe 2 testified that Minden Hospital was “a smaller hospital,” “very close to the [geographic] limits,” where he “[did]n’t really know anyone.” *Id.*, at 454. He applied to those hospitals where he believed he had the highest likelihood of success. *Ibid.* Given this evidence, the Fifth Circuit was wrong to conclude that the District Court’s findings in respect to Doe 2 were “clearly erroneous.” See *Anderson*, 470 U. S., at 575.

Doe 5

The District Court found that Doe 5 was unable to obtain admitting privileges at three hospitals in range of his Baton Rouge clinic in spite of his good-faith efforts to satisfy each hospital’s requirement that he find a covering physician. 250 F. Supp. 3d, at 76; see App. 1334–1335 (Women’s Hospi-

Opinion of BREYER, J.

tal); Record 2953 (Baton Rouge General), 10659–10661 (Lane Regional). The Court of Appeals disagreed. It thought that Doe 5's efforts reflected a "lackluster approach" because he asked only one doctor to cover him. 905 F. 3d, at 809.

The record shows, however, that Doe 5 asked the doctor most likely to respond affirmatively: the doctor with whom Doe 5's Baton Rouge clinic already had a patient transfer agreement. App. 1135. Yet Doe 5 testified that even this doctor was "too afraid to be my covering physician at the hospital" because, while the transfer agreement could apparently be "kept confidential," he feared that an agreement to serve as a covering physician would not remain a secret. *Id.*, at 1135–1136. And, if the matter became well known, the doctor whom Doe 5 asked worried that it could make him a target of threats and protests. *Ibid.*

Doe 5 was familiar with the problem. Anti-abortion protests had previously forced him to leave his position as a staff member of a hospital northeast of Baton Rouge. *Id.*, at 1137–1138, 1330. And activists had picketed the school attended by the children of a former colleague, who then stopped performing abortions as a result. Record 14036–14037.

With his own experience and their existing relationship in mind, Doe 5 could have reasonably thought that, if this doctor wouldn't serve as his covering physician, no one would. And it was well within the District Court's discretion to credit that reading of the record. Cf. *Cooper*, 581 U.S., at 293. Doe 5's testimony was internally consistent and consistent with what the District Court called the "mountain of un-contradicted and un-objected to evidence" in the record that supported its general finding "that opposition to abortion can present a major, if not insurmountable hurdle, for an applicant getting the required covering physician," including Doe 3's similar experience. 250 F. Supp. 3d, at 51, 49; see *id.*, at 51–53; App. 200–202.

Opinion of BREYER, J.

The Court of Appeals did not address this general finding or the evidence the District Court relied on to support it, and neither do our dissenting colleagues. Cf. *post*, at 396–397 (opinion of ALITO, J.); *post*, at 420 (opinion of GORSUCH, J.). The Court of Appeals pointed to what it described as Doe 4’s testimony that “finding a covering physician is not overly burdensome.” 905 F. 3d, at 809. Doe 4’s actual testimony was that he did not believe requiring doctors to obtain a covering physician was “an overburdensome requirement for admitting privileges.” Record 14154. In context, that statement is most naturally read as saying that such a requirement was reasonable, not that it was easy to fulfill. In fact, Doe 4 testified that he had been unable to apply to two hospitals for admitting privileges because he could not find a covering physician. *Id.*, at 14154–14155. Moreover, Doe 4’s statement referred to his efforts to obtain admitting privileges in New Orleans, not in Baton Rouge. *Ibid.* Doe 5 testified that he could more easily find a covering physician in New Orleans (where he did obtain privileges) because attitudes toward abortion there were less hostile than in Baton Rouge, so the doctors’ testimony would be consistent even under the Fifth Circuit’s view. App. 1335–1336. Once again, the appeals court’s conclusion cannot be squared with the standard of review. Cf. *Anderson*, 470 U. S., at 575.

Doe 6

Finally, the District Court found that, notwithstanding his good-faith efforts, Doe 6 would not be able to obtain admitting privileges within 30 miles of the clinic in New Orleans where he worked. The Court of Appeals did not question Doe 6’s decision not to apply to Tulane Hospital. Nor did it take issue with the District Court’s finding that his application to East Jefferson Hospital had been denied *de facto* through no fault of his own. 250 F. Supp. 3d, at 77; App. 54. But the appeals court reversed the District Court’s finding

Opinion of BREYER, J.

on the ground that Doe 6 should have (but did not) apply for admitting privileges at seven other hospitals in New Orleans, including Touro Hospital, which had granted limited privileges to Doe 5. 905 F. 3d, at 809–810.

Doe 6 testified that he did not apply to other hospitals because he did not admit a sufficient number of patients to receive active admitting privileges. App. 1310. As we have explained, *supra*, at 326–327, Doe 6 provides only medication abortions involving no surgical intervention. See App. 1308. The *State's* own admitting-privileges expert, Dr. Robert Marier, testified that a doctor in Doe 6's position would “probably not” be able to obtain “active admitting and surgical privileges” at *any* hospital. *Id.*, at 884; see 250 F. Supp. 3d, at 44 (finding Dr. Marier “generally well qualified” to express an opinion on “the issue of admitting privileges and hospital credentialing”).

The record contains the bylaws of four of the seven hospitals to which the Court of Appeals referred. All four directly support the testimony of Doe 6 and the *State's* expert. Three hospitals require doctors who receive admitting privileges to undergo a process of “focused professional practice evaluation.” See Record 2635, 2637, 2681 (Touro Hospital), 9054 (New Orleans East Hospital), 10755 (East Jefferson Hospital). As we have explained, this evaluation requires hospital staff to observe a doctor with admitting privileges while he or she performs a certain number of procedures. See *supra*, at 326–327. If the doctor admits no patients (and Doe 6 has no patients requiring admission), there is nothing to observe. Another hospital requires physicians to admit a minimum number of patients, either initially or after receiving admitting privileges. Record 9150–9153 (West Jefferson Hospital). And one requires both. *Id.*, at 9040, 9069 (New Orleans East Hospital). The record apparently is silent as to the remaining three hospitals, but that silence cannot contradict the well-supported testimony of Doe 6 and the *State's* expert that Doe 6 would not receive admitting privileges

Opinion of BREYER, J.

from any of them. Good faith does not require an exercise in futility.

We recognize that Doe 5 was able to secure limited admitting privileges at Touro Hospital, to which Doe 6 did not apply. But, unlike Doe 6, Doe 5 primarily performs surgical abortions. App. 1330. And while Doe 5 was a hospital-based physician as recently as 2012, Doe 6 has not held privileges at any hospital since 2005. *Id.*, at 1310, 1329. Doe 5's success therefore does not directly contradict the evidence that we have described in respect to Doe 6 or render the District Court's conclusion as to Doe 6 clearly erroneous. And, as we have said, "[a] finding that is 'plausible' in light of the full record—even if another is equally or more so—must govern." *Cooper*, 581 U. S., at 293.

Without actually disputing any of the evidence we have discussed, JUSTICE ALITO maintains that the plaintiffs could have introduced still more evidence to support the District Court's determination. See *post*, at 396. As we have said, however, "the trial on the merits should be 'the "main event" . . . rather than a "tryout on the road."'" *Anderson*, 470 U. S., at 575. "[T]he parties to a case on appeal have already been forced to concentrate their energies and resources on persuading the trial judge that their account of the facts is the correct one; requiring them to persuade three more judges at the appellate level—let alone another nine in this Court—"is requiring too much." *Ibid.*

Other Doctors

Finally, JUSTICE ALITO and JUSTICE GORSUCH suggest that the District Court failed to account for the possibility that new abortion providers might eventually replace Does 1, 2, 3, 5, and 6. See *post*, at 387–388 (opinion of ALITO, J.); *post*, at 419–421 (opinion of GORSUCH, J.). But the Court of Appeals did not dispute, and the record supports, the District Court's additional finding that, for "the same reasons that Does 1, 2, 4, 5, and 6 have had difficulties getting active

Opinion of BREYER, J.

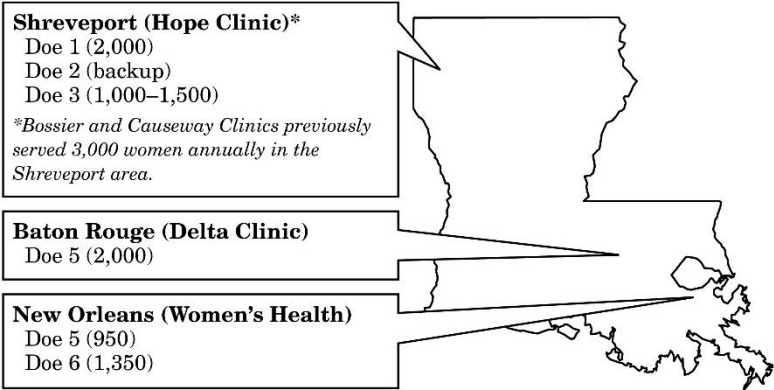
admitting privileges, reasons unrelated to their competence, . . . it is unlikely that the [a]ffected clinics will be able to comply with the Act by recruiting new physicians who have or can obtain admitting privileges.” 250 F. Supp. 3d, at 82.

B

Act 620’s Impact on Abortion Access

The District Court drew from the record evidence, including the factual findings we have just discussed, several conclusions in respect to the burden that Act 620 is likely to impose upon women’s ability to access abortions in Louisiana. To better understand the significance of these conclusions, the reader should keep in mind the geographic distribution of the doctors and their clinics. Figure 1 shows the distribution of doctors and clinics at the time of the District Court’s decision. Figure 2 shows the projected distribution if the admitting-privileges requirement were enforced, as found by the District Court. The figures in parentheses indicate the approximate number of abortions each physician performed annually, according to the District Court.

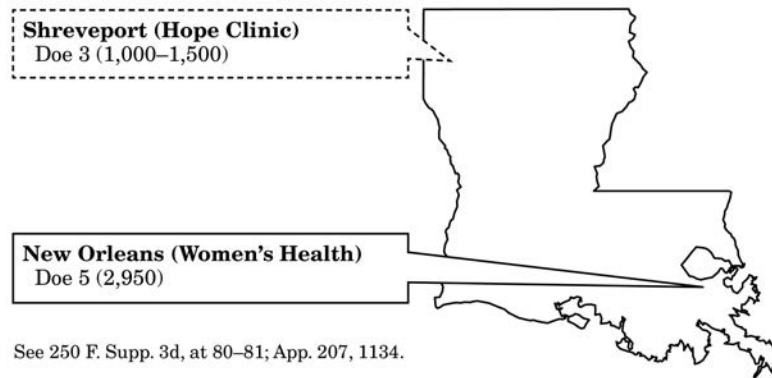
Figure 1 — Distribution of Abortion Clinics and Providers at the Time of the District Court’s Decision



See 250 F. Supp. 3d, at 40–41; App. 1122, 1125, 1134, 1138, 1141, 1256.

Opinion of BREYER, J.

Figure 2 — Projected Distribution of Abortion Clinics and Providers Following Enforcement of Act 620



See 250 F. Supp. 3d, at 80–81; App. 207, 1134.

1

As we have seen, enforcing the admitting-privileges requirement would eliminate Does 1, 2, and 6. The District Court credited Doe 3's uncontradicted, in-court testimony that he would stop performing abortions if he was the last provider in northern Louisiana. 250 F. Supp. 3d, at 79; see App. 263–265. So the departure of Does 1 and 2 would also eliminate Doe 3. That would leave only Doe 5. And Doe 5's inability to obtain privileges in the Baton Rouge area would leave Louisiana with just one clinic with one provider to serve the 10,000 women annually who seek abortions in the State. 250 F. Supp. 3d, at 80, 87–88; cf. *Whole Woman's Health*, 579 U. S., at 613–614.

Working full time in New Orleans, Doe 5 would be able to absorb no more than about 30% of the annual demand for abortions in Louisiana. App. 1134, 1331; see *id.*, at 1129. And because Doe 5 does not perform abortions beyond 18 weeks, women between 18 weeks and the state legal limit of 20 weeks would have little or no way to exercise their constitutional right to an abortion. *Id.*, at 1330–1331.

Those women not altogether prevented from obtaining an abortion would face other burdens. As in *Whole Woman's*

Opinion of BREYER, J.

Health, the reduction in abortion providers caused by Act 620 would inevitably mean “longer waiting times, and increased crowding.” 579 U.S., at 613. The District Court heard testimony that delays in obtaining an abortion increase the risk that a woman will experience complications from the procedure and may make it impossible for her to choose a noninvasive medication abortion. App. 220, 290, 312–313; see also *id.*, at 1139, 1305, 1313, 1316, 1323.

Even if they obtain an appointment at a clinic, women who might previously have gone to a clinic in Baton Rouge or Shreveport would face increased driving distances. New Orleans is nearly a five hour drive from Shreveport; it is over an hour from Baton Rouge; and Baton Rouge is more than four hours from Shreveport. The impact of those increases would be magnified by Louisiana’s requirement that every woman undergo an ultrasound and receive mandatory counseling at least 24 hours before an abortion. La. Rev. Stat. Ann. § 40:1061.10(D). A Shreveport resident seeking an abortion who might previously have obtained care at one of that city’s local clinics would either have to spend nearly 20 hours driving back and forth to Doe 5’s clinic twice, or else find overnight lodging in New Orleans. As the District Court stated, both experts and laypersons testified that the burdens of this increased travel would fall disproportionately on poor women, who are least able to absorb them. App. 106–107, 178, 502–508, 543; see also *id.*, at 311–312.

2

We note that the Court of Appeals also faulted the District Court for factoring Doe 3’s departure into its calculations. The appeals court thought that Doe 3’s personal choice to stop practicing could not be attributed to Act 620. 905 F.3d, at 810–811. That is beside the point. Even if we pretended as though (contrary to the record evidence) Doe 3 would continue to provide abortions at Shreveport-based Hope Clinic, the record nonetheless supports the District Court’s alterna-

Opinion of BREYER, J.

tive finding that Act 620's burdens would remain substantial. See 250 F. Supp. 3d, at 80–81, 84, 87.

The record tells us that Doe 3 is presently able to see roughly 1,000–1,500 women annually. *Id.*, at 81; see App. 207, 243–244. Doe 3 testified that this was in addition to “working very, very long hours maintaining [his] private [OB/GYN] practice.” *Id.*, at 265, 1323; see *id.*, at 118, 1147. And, the District Court found that Doe 5 can perform no more than roughly 3,000 abortions annually. See *supra*, at 337. So even if Doe 3 remained active in Shreveport, the annual demand for abortions in Louisiana would be more than double the capacity. And although the availability of abortions in Shreveport might lessen the driving distances faced by some women, it would still leave thousands of Louisiana women with no practical means of obtaining a safe, legal abortion, and it would not meaningfully address the health risks associated with crowding and delay for those able to secure an appointment with one of the State's two remaining providers.

* * *

Taken together, we think that these findings and the evidence that underlies them are sufficient to support the District Court's conclusion that Act 620 would place substantial obstacles in the path of women seeking an abortion in Louisiana.

V

Benefits

We turn finally to the law's asserted benefits. The District Court found that there was “‘no significant health-related problem that the new law helped to cure.’” 250 F. Supp. 3d, at 86 (quoting *Whole Woman's Health*, 579 U. S., at 610). It found that the admitting-privileges requirement “[d]oes [n]ot [p]rotect [w]omen's [h]ealth,” provides “no significant health benefits,” and makes no improvement to women's health “compared to prior law.” 250 F. Supp. 3d,

Opinion of BREYER, J.

at 86 (boldface deleted). Our examination of the record convinces us that these findings are not “clearly erroneous.”

First, the District Court found that the admitting-privileges requirement serves no “relevant credentialing function.” *Id.*, at 87 (quoting *Whole Woman’s Health*, 579 U.S., at 613). As we have seen, hospitals can, and do, deny admitting privileges for reasons unrelated to a doctor’s ability safely to perform abortions. And Act 620’s requirement that physicians obtain privileges at a hospital within 30 miles of the place where they perform abortions further constrains providers for reasons that bear no relationship to competence.

Moreover, while “competency is a factor” in credentialing decisions, 250 F. Supp. 3d, at 46, hospitals primarily focus upon a doctor’s ability to perform the inpatient, hospital-based procedures for which the doctor seeks privileges—not outpatient abortions. App. 877, 1373; see *id.*, at 907; Brief for Medical Staff Professionals as *Amici Curiae* 26; Brief for American College of Obstetricians and Gynecologists et al. as *Amici Curiae* 12. Indeed, the State’s admitting-privileges expert, Dr. Robert Marier, testified that, when he served as the Executive Director of Louisiana’s Board of Medical Examiners, he concurred in the Board’s position that a physician was competent to perform first-trimester surgical abortions and to “recognize and address complications from the procedure” so long as they had completed an accredited residency in obstetrics and gynecology or been trained in abortion procedures during another residency—irrespective of their affiliation with any hospital. App. 872–873, 1305; cf. *post*, at 381–382 (ALITO, J., dissenting). And nothing in the record indicates that the background vetting for admitting privileges adds significantly to the vetting that the State Board of Medical Examiners already provides. 250 F. Supp. 3d, at 87; App. 1355–1356, 1358–1359.

Second, the District Court found that the admitting-privileges requirement “does not conform to prevailing medi-

Opinion of BREYER, J.

cal standards and will not improve the safety of abortion in Louisiana.” 250 F. Supp. 3d, at 64; see *id.*, at 64–66. As in *Whole Woman’s Health*, the expert and lay testimony presented at trial shows:

- “Complications from surgical abortion are relatively rare,” and “[t]hey very rarely require transfer to a hospital or emergency room and are generally not serious.” App. 287; see *id.*, at 129; cf. *Whole Woman’s Health*, 579 U. S., at 610–611.
- For those patients who do experience complications at the clinic, the transfer agreement required by existing law is “sufficient to ensure continuity of care for patients in an emergency.” App. 1050; see *id.*, at 194, 330–332, 1059.
- The “standard protocol” when a patient experiences a complication after returning home from the clinic is to send her “to the hospital that is nearest and able to provide the service that the patient needs,” which is not necessarily a hospital within 30 miles of the clinic. *Id.*, at 351; see *id.*, at 115–116, 180, 793; La. Rev. Stat. Ann. § 40:1061.10(A)(2)(b)(ii) (requiring abortion providers to furnish patients with the name and telephone number of the hospital nearest to their home); cf. *Whole Woman’s Health*, 579 U. S., at 611.

As in *Whole Woman’s Health*, the State introduced no evidence “showing that patients have better outcomes when their physicians have admitting privileges” or “of any instance in which an admitting privileges requirement would have helped even one woman obtain better treatment.” 250 F. Supp. 3d, at 64; *Whole Woman’s Health*, 579 U. S., at 611–612; see also Centers for Medicare and Medicaid Services, 84 Fed. Reg. 51790–51791 (2019) (“Under modern procedures, emergency responders (and patients themselves) take patients to hospital emergency rooms without regard to prior agreements between particular physicians and particu-

Opinion of BREYER, J.

lar hospitals”); Brief for American College of Obstetricians and Gynecologists et al. as *Amici Curiae* 6 (local admitting-privileges requirements for abortion providers offer no medical benefit and do not meaningfully advance continuity of care).

VI

Conclusion

We conclude, in light of the record, that the District Court’s significant factual findings—both as to burdens and as to benefits—have ample evidentiary support. None is “clearly erroneous.” Given the facts found, we must also uphold the District Court’s related factual and legal determinations. These include its determination that Louisiana’s law poses a “substantial obstacle” to women seeking an abortion; its determination that the law offers no significant health-related benefits; and its determination that the law consequently imposes an “undue burden” on a woman’s constitutional right to choose to have an abortion. We also agree with its ultimate legal conclusion that, in light of these findings and our precedents, Act 620 violates the Constitution.

VII

As a postscript, we explain why we have found unconvincing several further arguments that the State has made. First, the State suggests that the record supports the Court of Appeals’ conclusion that Act 620 poses no substantial obstacle to the abortion decision. See Brief for Respondent 73, 80. This argument misconceives the question before us. “The question we must answer” is “not whether the [Fifth] Circuit’s interpretation of the facts was clearly erroneous, but whether the *District Court’s* finding[s] were] clearly erroneous.” *Anderson*, 470 U. S., at 577 (emphasis added). As we have explained, we think the District Court’s factual findings here are plausible in light of the record as a whole.

Opinion of BREYER, J.

Nothing in the State’s briefing furnishes a basis to disturb that conclusion.

Second, the State says that the record does not show that Act 620 will burden *every* woman in Louisiana who seeks an abortion. Brief for Respondent 69–70 (citing *United States v. Salerno*, 481 U. S. 739, 745 (1987)). True, but beside the point. As we stated in *Casey*, a State’s abortion-related law is unconstitutional on its face if “it will operate as a substantial obstacle to a woman’s choice to undergo an abortion” in “a large fraction of the cases in which [it] is relevant.” 505 U. S., at 895 (majority opinion). In *Whole Woman’s Health*, we reaffirmed that standard. We made clear that the phrase refers to a large fraction of “those women for whom the provision is an actual rather than an irrelevant restriction.” 579 U. S., at 626 (quoting *Casey*, 505 U. S., at 895; brackets omitted). That standard, not an “every woman” standard, is the standard that must govern in this case.

Third, the State argues that Act 620 would not make it “nearly impossible” for a woman to obtain an abortion. Brief for Respondent 71–72. But, again, the words “nearly impossible” do not describe the legal standard that governs here. Since *Casey*, we have repeatedly reiterated that the plaintiff’s burden in a challenge to an abortion regulation is to show that the regulation’s “purpose or effect” is to “plac[e] a substantial obstacle in the path of a woman seeking an abortion of a nonviable fetus.” 505 U. S., at 877 (plurality opinion); see *Whole Woman’s Health*, 579 U. S., at 595–596; *Gonzales*, 550 U. S., at 156; *Stenberg*, 530 U. S., at 921; *Mazurek*, 520 U. S., at 971.

Finally, the State makes several arguments about the standard of review that it would have us apply in cases where a regulation is found *not* to impose a substantial obstacle to a woman’s choice. Brief for Respondent 60–66. That, however, is not this case. The record here establishes

ROBERTS, C. J., concurring in judgment

that Act 620's admitting-privileges requirement places a substantial obstacle in the path of a large fraction of those women seeking an abortion for whom it is a relevant restriction.

* * *

This case is similar to, nearly identical with, *Whole Woman's Health*. And the law must consequently reach a similar conclusion. Act 620 is unconstitutional. The Court of Appeals' judgment is erroneous. It is

Reversed.

CHIEF JUSTICE ROBERTS, concurring in judgment.

In July 2013, Texas enacted a law requiring a physician performing an abortion to have "active admitting privileges at a hospital . . . located not further than 30 miles from the location at which the abortion is performed." Tex. Health & Safety Code Ann. § 171.0031(a)(1)(A) (West Cum. Supp. 2019). The law caused the number of facilities providing abortions to drop in half. In *Whole Woman's Health v. Hellerstedt*, 579 U. S. 582 (2016), the Court concluded that Texas's admitting privileges requirement "places a substantial obstacle in the path of women seeking a previability abortion" and therefore violated the Due Process Clause of the Fourteenth Amendment. *Id.*, at 591 (citing *Planned Parenthood of Southeastern Pa. v. Casey*, 505 U. S. 833, 878 (1992) (plurality opinion)).

I joined the dissent in *Whole Woman's Health* and continue to believe that the case was wrongly decided. The question today however is not whether *Whole Woman's Health* was right or wrong, but whether to adhere to it in deciding the present case. See *Moore v. Texas*, 586 U. S. 133, 143 (2019) (ROBERTS, C. J., concurring).

Today's case is a challenge from several abortion clinics and providers to a Louisiana law nearly identical to the Texas law struck down four years ago in *Whole Woman's Health*. Just like the Texas law, the Louisiana law requires

ROBERTS, C. J., concurring in judgment

physicians performing abortions to have “active admitting privileges at a hospital . . . located not further than thirty miles from the location at which the abortion is performed.” La. Rev. Stat. Ann. § 40:1061.10(A)(2)(a) (West Cum. Supp. 2020). Following a six-day bench trial, the District Court found that Louisiana’s law would “result in a drastic reduction in the number and geographic distribution of abortion providers.” *June Medical Services LLC v. Kliebert*, 250 F. Supp. 3d 27, 87 (MD La. 2017). The law would reduce the number of clinics from three to “one, or at most two,” and the number of physicians providing abortions from five to “one, or at most two,” and “therefore cripple women’s ability to have an abortion in Louisiana.” *Id.*, at 87–88.

The legal doctrine of *stare decisis* requires us, absent special circumstances, to treat like cases alike. The Louisiana law imposes a burden on access to abortion just as severe as that imposed by the Texas law, for the same reasons. Therefore Louisiana’s law cannot stand under our precedents.

I

Stare decisis (“to stand by things decided”) is the legal term for fidelity to precedent. Black’s Law Dictionary 1696 (11th ed. 2019). It has long been “an established rule to abide by former precedents, where the same points come again in litigation; as well to keep the scale of justice even and steady, and not liable to waver with every new judge’s opinion.” 1 W. Blackstone, Commentaries on the Laws of England 69 (1765). This principle is grounded in a basic humility that recognizes today’s legal issues are often not so different from the questions of yesterday and that we are not the first ones to try to answer them. Because the “private stock of reason . . . in each man is small, . . . individuals would do better to avail themselves of the general bank and capital of nations and of ages.” 3 E. Burke, Reflections on the Revolution in France 110 (1790).

ROBERTS, C. J., concurring in judgment

Adherence to precedent is necessary to “avoid an arbitrary discretion in the courts.” The Federalist No. 78, p. 529 (J. Cooke ed. 1961) (A. Hamilton). The constraint of precedent distinguishes the judicial “method and philosophy from those of the political and legislative process.” Jackson, *Decisional Law and Stare Decisis*, 30 A. B. A. J. 334 (1944).

The doctrine also brings pragmatic benefits. Respect for precedent “promotes the evenhanded, predictable, and consistent development of legal principles, fosters reliance on judicial decisions, and contributes to the actual and perceived integrity of the judicial process.” *Payne v. Tennessee*, 501 U. S. 808, 827 (1991). It is the “means by which we ensure that the law will not merely change erratically, but will develop in a principled and intelligible fashion.” *Vasquez v. Hillery*, 474 U. S. 254, 265 (1986). In that way, “*stare decisis* is an old friend of the common lawyer.” Jackson, *supra*, at 334.

Stare decisis is not an “inexorable command.” *Ramos v. Louisiana*, 590 U. S. 83, 105 (2020) (internal quotation marks omitted). But for precedent to mean anything, the doctrine must give way only to a rationale that goes beyond whether the case was decided correctly. The Court accordingly considers additional factors before overruling a precedent, such as its administrability, its fit with subsequent factual and legal developments, and the reliance interests that the precedent has engendered. See *Janus v. State, County, and Municipal Employees*, 585 U. S. 878, 917 (2018).

Stare decisis principles also determine how we handle a decision that itself departed from the cases that came before it. In those instances, “[r]emaining true to an ‘intrinsically sounder’ doctrine established in prior cases better serves the values of *stare decisis* than would following” the recent departure. *Adarand Constructors, Inc. v. Peña*, 515 U. S. 200, 231 (1995) (plurality opinion). *Stare decisis* is pragmatic and contextual, not “a mechanical formula of adherence to the latest decision.” *Helvering v. Hallock*, 309 U. S. 106, 119 (1940).

ROBERTS, C. J., concurring in judgment

II

A

Both Louisiana and the providers agree that the undue burden standard announced in *Casey* provides the appropriate framework to analyze Louisiana’s law. Brief for Petitioners in No. 18–1323, pp. 45–47; Brief for Respondent in No. 18–1323, pp. 60–62. Neither party has asked us to reassess the constitutional validity of that standard.

Casey reaffirmed “the most central principle of *Roe v. Wade*,” a “woman’s right to terminate her pregnancy before viability.” *Casey*, 505 U. S., at 871 (plurality opinion).¹ At the same time, it recognized that the State has “important and legitimate interests in . . . protecting the health of the pregnant woman and in protecting the potentiality of human life.” *Id.*, at 875–876 (internal quotation marks and brackets omitted).

To serve the former interest, the State may, “[a]s with any medical procedure,” enact “regulations to further the health or safety of a woman seeking an abortion.” *Id.*, at 878. To serve the latter interest, the State may, among other things, “enact rules and regulations designed to encourage her to know that there are philosophic and social arguments of great weight that can be brought to bear in favor of continuing the pregnancy to full term.” *Id.*, at 872. The State’s freedom to enact such rules is “consistent with *Roe*’s central premises, and indeed the inevitable consequence of our holding that the State has an interest in protecting the life of the unborn.” *Id.*, at 873.

Under *Casey*, the State may not impose an undue burden on the woman’s ability to obtain an abortion. “A finding of an undue burden is a shorthand for the conclusion that a

¹ Although parts of *Casey*’s joint opinion were a plurality not joined by a majority of the Court, the joint opinion is nonetheless considered the holding of the Court under *Marks v. United States*, 430 U. S. 188, 193 (1977), as the narrowest position supporting the judgment.

ROBERTS, C. J., concurring in judgment

state regulation has the purpose or effect of placing a substantial obstacle in the path of a woman seeking an abortion of a nonviable fetus.” *Id.*, at 877. Laws that do not pose a substantial obstacle to abortion access are permissible, so long as they are “reasonably related” to a legitimate state interest. *Id.*, at 878.

After faithfully reciting this standard, the Court in *Whole Woman’s Health* added the following observation: “The rule announced in *Casey* . . . requires that courts consider the burdens a law imposes on abortion access together with the benefits those laws confer.” 579 U. S., at 607. The plurality repeats today that the undue burden standard requires courts “to weigh the law’s asserted benefits against the burdens it imposes on abortion access.” *Ante*, at 306–307 (internal quotation marks omitted).

Read in isolation from *Casey*, such an inquiry could invite a grand “balancing test in which unweighted factors mysteriously are weighed.” *Marrs v. Motorola, Inc.*, 577 F.3d 783, 788 (CA7 2009). Under such tests, “equality of treatment is . . . impossible to achieve; predictability is destroyed; judicial arbitrariness is facilitated; judicial courage is impaired.” Scalia, *The Rule of Law as a Law of Rules*, 56 U. Chi. L. Rev. 1175, 1182 (1989).

In this context, courts applying a balancing test would be asked in essence to weigh the State’s interests in “protecting the potentiality of human life” and the health of the woman, on the one hand, against the woman’s liberty interest in defining her “own concept of existence, of meaning, of the universe, and of the mystery of human life” on the other. *Casey*, 505 U.S., at 851 (opinion of the Court); *id.*, at 871 (plurality opinion) (internal quotation marks omitted). There is no plausible sense in which anyone, let alone this Court, could objectively assign weight to such imponderable values and no meaningful way to compare them if there were. Attempting to do so would be like “judging whether a particular line is longer than a particular rock is heavy,”

ROBERTS, C. J., concurring in judgment

Bendix Autolite Corp. v. Midwesco Enterprises, Inc., 486 U. S. 888, 897 (1988) (Scalia, J., concurring in judgment). Pretending that we could pull that off would require us to act as legislators, not judges, and would result in nothing other than an “unanalyzed exercise of judicial will” in the guise of a “neutral utilitarian calculus.” *New Jersey v. T. L. O.*, 469 U. S. 325, 369 (1985) (Brennan, J., concurring in part and dissenting in part).

Nothing about *Casey* suggested that a weighing of costs and benefits of an abortion regulation was a job for the courts. On the contrary, we have explained that the “traditional rule” that “state and federal legislatures [have] wide discretion to pass legislation in areas where there is medical and scientific uncertainty” is “consistent with *Casey*.” *Gonzales v. Carhart*, 550 U. S. 124, 163 (2007). *Casey* instead focuses on the existence of a substantial obstacle, the sort of inquiry familiar to judges across a variety of contexts. See, e. g., *Burwell v. Hobby Lobby Stores, Inc.*, 573 U. S. 682, 694–695 (2014) (asking whether the government “substantially burdens a person’s exercise of religion” under the Religious Freedom Restoration Act); *Arizona Free Enterprise Club’s Freedom Club PAC v. Bennett*, 564 U. S. 721, 748 (2011) (asking whether a law “imposes a substantial burden on the speech of privately financed candidates and independent expenditure groups”); *Murphy v. United Parcel Service, Inc.*, 527 U. S. 516, 521 (1999) (asking, in the context of the Americans with Disabilities Act, whether an individual’s impairment “substantially limits one or more major life activities” (internal quotation marks omitted)).

Casey’s analysis of the various restrictions that were at issue in that case is illustrative. For example, the opinion-recognized that Pennsylvania’s 24-hour waiting period for abortions “has the effect of increasing the cost and risk of delay of abortions,” but observed that the District Court did not find that the “increased costs and potential delays amount to substantial obstacles.” 505 U. S., at 886 (joint

ROBERTS, C. J., concurring in judgment

opinion of O'Connor, Kennedy, and Souter, JJ.) (internal quotation marks omitted). The opinion concluded that "given the statute's definition of medical emergency," the waiting period did not "impose[] a real health risk." *Ibid.* Because the law did not impose a substantial obstacle, *Casey* upheld it. And it did so notwithstanding the District Court's finding that the law did "not further the state interest in maternal health." *Ibid.* (internal quotation marks omitted).

Turning to the State's various recordkeeping and reporting requirements, *Casey* found those requirements do not "impose a substantial obstacle to a woman's choice" because "[a]t most they might increase the cost of some abortions by a slight amount." *Id.*, at 901. "While at some point increased cost could become a substantial obstacle," there was "no such showing on the record" before the Court. *Ibid.* The Court did not weigh this cost against the benefits of the law.

The same was true for Pennsylvania's parental consent requirement. *Casey* held that "a State may require a minor seeking an abortion to obtain the consent of a parent or guardian, provided that there is an adequate judicial bypass procedure." *Id.*, at 899 (citing, among other cases, *Ohio v. Akron Center for Reproductive Health*, 497 U. S. 502, 510–519 (1990)). *Casey* relied on precedent establishing that judicial bypass procedures "prevent another person from having an absolute veto power over a minor's decision to have an abortion." *Akron*, 497 U. S., at 510. Without a judicial bypass, parental consent laws impose a substantial obstacle to a minor's ability to obtain an abortion and therefore constitute an undue burden. See *Casey*, 505 U. S., at 899 (joint opinion).

The opinion similarly looked to whether there was a substantial burden, not whether benefits outweighed burdens, in analyzing Pennsylvania's requirement that physicians provide certain "truthful, nonmisleading information" about the nature of the abortion procedure. *Id.*, at 882. The opinion